

Discover brief

Ambient Clinical Value Chain Activation

Tenant: Meridian Health System

Tenant key: meridian-health

Move ref: meridian:move:ambient-clinical-value-chain-activation

Generated: 2026-05-20

Problem statement, current-state baseline (with source and seed gaps), opportunity sizing and the go/no-go read.

Every figure in this document is produced by the Moves Expert Kernel from the tenant's audited substrate. Absent data is declared a seed gap, never invented. Effort rests on a researched planning rate card — a market range, not a quote. Generated by AbarVa · Moves.

Discover — problem, baseline, opportunity, go/no-go

Ambient Clinical Value Chain Activation · meridian-health

Discover establishes the current state honestly before any solution is shaped. The opportunity is bounded by what the recorded baseline supports; declared seed gaps cap it.

Current-state baseline

Metric	Value	Source	Quality
Primary-care pajama time (after-hours Epic)	129 minutes_per_day	Evidence base E17 (D06 Documentation-Time Baseline · specialty decomposition)	measured/high
MBI-HSS emotional-exhaustion burnout (elevated band)	54 percent	Evidence base E2 (D06 · MBI-HSS burnout survey wave, n=1,240)	measured/high
HCC capture rate on the MA panel	58 percent	Evidence base E28 (D10 Benchmark Comparison · cohort scorecard)	measured/medium
Note turnaround (sign-to-close)	11.4 hours	Evidence base E8 (D03 Success Metric Tree · supporting metric)	measured/medium
CDI query resolution cycle time	38 hours	Evidence base E8 (D03 Success Metric Tree · CDI cycle-time metric)	measured/medium
CDI HCC-relevant query volume	40 queries_per_week	Evidence base E25 (D08/D10 · CDI-queue routing volume benchmark)	measured/medium
Quality-measure documentation capture (HEDIS + Stars)	34 percent	Evidence base E47 (D15 Intervention Portfolio · lever outcome envelope)	measured/medium
Clinician retention (annual)	88 percent	Evidence base E9 (D16 Business Case · cognitive-load recovery component)	stated/medium
Ambulatory visits per clinician	84 visits_per_clinician_per_week	Evidence base E19 (D04 Intake · chief ambulatory officer field shadow)	stated/medium
Cost-per-clinician-hour conversion coefficient	Not recorded — seed gap	seed gap	absent/low
RAF-to-revenue conversion coefficient	Not recorded — seed gap	seed gap	absent/low
Locum-avoidance dollar basis	Not recorded — seed gap	seed gap	absent/low

Seed gaps — declared, not fabricated

- Cost-per-clinician-hour conversion coefficient: Not measured. The value-at-stake model (E9) converts recovered pajama time into dollars, but the fully-loaded cost-per-clinician-hour basis is a modeled assumption — Meridian Finance has not attested a figure. Needed to monetise cognitive-load recovery.
- RAF-to-revenue conversion coefficient: Not measured. HCC recapture is sized at \$3.2–5.1M/yr (E5/E9), but the RAF-point-to-revenue coefficient on the MA book is a Revenue-Cycle estimate, not an attested baseline. Needed to monetise HCC capture lift.
- Locum-avoidance dollar basis: Not measured. The cognitive-load recovery component (E9) names locum avoidance as a value lever, but no locum-spend baseline or avoidance rate is seeded. Modeled, not measured.

Opportunity & go/no-go read

Verdict: SHAPE — do not fund as-is

The critic raised 1 blocker(s) — fund only after they are resolved (see critic findings). Re-shape the Move to close them, do not fund as-is.

The case breaks if the baseline seed gap is not closed — without the missing unit economics the value range is a proxy, not a forecast.

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